

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : نَعْمَهُ صَبْحَى شَلْبَى مُحَمَّد

الحى / المدينة : الساحل

المحافظة : القاهرة

رقم البطاقة : 28501031401423 رقم التلفون : 01142555601

العمر (تاريخ الميلاد) : 03-01-1985 المهنة : لاتعمل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

GR1-E
جورستان
روستای پلانی
لندی حسن
موسین رمضان
صحة البیابان المرفقة
للجنة و مدین

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة الايام المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --
ملاحظات:

ملحوظات : ---

قسم :

نفقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

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Name: نعمه صبحي شلبي محمد National ID: 28501031401423	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نعمه
صبحي شلبي محمد
٢ ش عوض ابوسعد
المناحل - القاهرة

١٩٨٥/٠١/١١ ٢٨٥٠١٠٣١٤٠١٤٢٣

LU2585074

٢٠٢٦/٠٥/١١ (٢٨٥٠١) ٢٨٥٠١٠٣١٤٠١٤٢٣

ربة منزل
أنثى
عز حمدي محمد رزق
البطاقة سارية حتى ٢٠٢٣/٠٥/١١

١



Ministry of Health and Prevention
Kingdom of Saudi Arabia

وزارة الصحة والوقاية
المملكة العربية السعودية



A-Patient ID:

1- Name: محمد بن علي محمد

Sex:

MDT form

2- Medical number:

Female

03163214

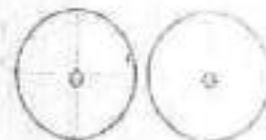
Old Med:

Name:

3492-2023

3-ID number: 28501021191423

Patient Age: 49 Y



Family history:

Family history:

Phone:

01294457349

WH:

WH:

REASON OF MDT:

Reason of MDT:

Marital status:

Marital status:

RH Score:

RH Score:

GROUP:

GROUP:

Date of MDT:

Date of MDT:

Co-morbidity:

Co-morbidity:

Nonopposed status:

Nonopposed status:

Initial STAGE:

Initial STAGE:

11/05/2024

Clinical data

Breast Imaging:

Mets Workup:
MBC to bone

BR	ER	PR	Her 2 Neu	SISH	HER7
IOC	0/0	3/3	3		20
HER2/NEU	HER2/NEU	HER2/NEU	HER2/NEU	SISH comment	
Y		N		M	1 Bone

Sonomammography

PATIENT HISTORY

started 27 cycles taxol+ 9 cycles herceptin last 17/4/2024-->clinical progression
ct: D12 collapse SD
BS: mild progression
pt received RTH to D12
New Bone scan showed PD
shifted to navelbine but pt had poor compliance to navelbine.

MDT OUTCOME & PLAN

Shift to Gemzar (full dose) / Tykerb (with dose reduction).

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

Name: نعمه صبحي شلبي محمد		
MR. : 1080		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient has left breast infiltrating duct carcinoma, NOS, staged cT4 N2. Initial therapy consisted of Taxol for 27 weeks followed by 9 cycles of Herceptin. Disease progression prompted a change to Navelbine and Tykerb for three cycles in August 2024, after which Gemzar was initiated on 21 October 2025. Imaging in May 2025 demonstrated stable disease, but by November 2025 there was radiographic progression with new bone lesions, leading to a multidisciplinary discussion recommending carboplatin and KADCYLA from an external provider; treatment was stopped around 14 November for reassessment and approval. The patient, weight 72 kg, height 160 cm, has metastatic left breast cancer involving bone. Pathology shows IDC grade 2, estrogen receptor 8/8, progesterone receptor 8/8, HER2 3+, Ki-67 70%. Prior therapy included Taxol for 27 weeks and 9 cycles of Herceptin, followed by Navelbine and Tykerb for three cycles, which were intolerable. Gemzar single-agent therapy was given from August to November 2024, with a best response of stable disease in December 2024, but subsequent imaging in May 2025 revealed progression of iliac bony lesions, D12 vertebral collapse, and L5 wedging. A multidisciplinary team considered carboplatin single-agent (planned for December 2025) and a trial of KADCYLA, but the patient has not yet received further treatment; new CT in April 2026 showed multiple osseous lesions in vertebral and iliac sites, and a carboplatin regimen at AUC 5 was planned pending dental evaluation and brain MRI. The patient received carboplatin at AUC 5 as cycle 2 with supportive therapy including cympatix, gabtin 300 mg, and paracetamol; laboratory results, brain MRI, and dental evaluation were pending. The patient has completed cycle 2 of carboplatin at AUC 5. Bone scan was free for cycle 3 of carboplatin at AUC 5; imaging confirmed multiple osseous lesions, and the patient was neutropenic, prompting a one-week postponement of treatment. The patient has completed cycle 3 of carboplatin at AUC 5. The patient reported severe neutropenia during cycle 5, week 1 of carboplatin at AUC 2.5, and continued with week 2 in the same cycle. Additional issues included a nipple infection treated with fusidin cream and administration of zoladex 10.8 mg; PET/CT was pending. The patient has completed cycle 5, week 1 and week 2 of carboplatin at AUC 2.5. The patient refused PET/CT and declined further carboplatin for reassessment by CT CAP and bone scan; a multidisciplinary team discussion was noted.		
تحريرا في ,		
	التاريخ : 21-09-2026	الوقت :
dina makram		الطبيب المعالج :
يعتمد ,		

Patient's Name: neama sobhy shalabi mohamed
Date: 2026-04-02
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Known case of breast cancer
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple scattered sclerotic lesions seen at the vertebral bodies, bilateral iliac bones and left clavicle

Opinion:

- ❖ Known case of breast cancer
- ❖ Multiple bony metastatic lesions as described

Dr.Asser Ahmed

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Dr.Asser Ahmed

Dr.Asser Ahmed

Dr.Asser Ahmed

Presidential Initiative for Women's Health
Pathology Lab - Cairo University Hospitals

المبادرة الرئاسية لصحة المرأة - مستشفى الباطن والجراح
مستشفيات جامعة القاهرة

PATHOLOGY REPORT

Name: أسماء محمد شبيب

Patient's I.D.: 28501031401423

Age: 38 Y.

Clinical data: Left breast locally advanced mass with diffuse skin thickening.
Nature of the specimen: U.S. Guided Tru Cut Biopsy.

Pathology Number: 351 /2023

Specimen Received Date: 30 / 08 /2023

Date Report Issued: 7 / 09 /2023

GROSS EXAMINATION: Tissue core biopsy, totally submitted.

MICROSCOPIC EXAMINATION: Serial sections examined revealed cores of malignant tumor formed of groups, cords and cribriform structures of moderately anaplastic ductal epithelial cells. Surrounding desmoplastic stroma with <5% TILs was seen. Tiny skin covered fragment was seen showing dermal proliferated capillaries with dense widely crushed inflammatory cellular infiltrate. Area of fat necrosis was also seen.

IMMUNOHISTOCHEMISTRY PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-Her-2/neu (1B5) Primary Antibody), and CONFIRM anti-Ki67 (30-2) Primary Antibody, using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

IMMUNOHISTOCHEMISTRY RESULTS:

Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.
Examination of section treated for progesterone receptors revealed strong positive nuclear staining in 70% of tumor cells by chromogen.
Examination of section treated for Her-2/neu revealed strong complete membrane staining in most of tumor cells by chromogen (score 3+).
Examination of section treated for Ki67 revealed positive nuclear stain in average of 70% of tumor cells.

DIAGNOSIS: Left Breast Mass, U.S. Guided Tru Cut Biopsy:

- Invasive Duct Carcinoma, NST, Grade II.
- **Positive** For Estrogen Receptors, Score 8/8.
- **Positive** For Progesterone Receptors, Score 8/8.
- **Positive** For Her-2/neu Expression (Score 3+).
- **70%** Ki-67 Proliferative index.

COMMENT: All controls show appropriate reactivity.

DR. SARA ABOU-FANDOUD
DR. PASSANT ESSAM SHIBEL

Patient Name: Ne'ma Sobhy Shalabi Mohamed

Exam: CT CAP+C (CT Chest Abd. Pelvis)

Medical ID:

Date: 13/10/2025

Age: 40 Year(s)

Pat. ID: 42376

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

- As Compared to the previous study dated 27/05/2025.

I Chest:

• Still noted rather unchanged:

- Left breast parenchymal distortion with diffuse skin thickening.
- Few subcentimetric prominent left axillary lymph nodes largest measuring 5mm in short axis.
- No sizable worrisome pulmonary masses, areas of consolidation or cavitary lesions.
- No sizable pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is still seen mildly enlarged with diffuse fatty infiltration with Stable size of the previously noted hepatic dome (Segment VIII) small hypodense focal lesion measures about 6 mm with peripheral nodular enhancement? hemangioma.
- No biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows:
 - Stable few lytic osseous lesions involving both iliac bones and vertebrae, with still noted D12 vertebral collapse & L5 central wedging.

Opinion:

- Follow-up status showing rather stationary course since the last study.

Signature Prof. Ali Remah
14/10/2025 05:20.



Exam Information:

<u>Patient Name:</u> Neama Sobhy	<u>Date of birth:</u> 30/10/1985
<u>Arabic Name:</u> نعمة صبحي	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 368863	<u>Exam Date:</u> 05/05/2025
<u>Patient HIS ID:</u> 10110225007430	
<u>Written by:</u> Dr. Hoda Pathy	<u>Approved by:</u> Dr. Hoda Pathy

^{99m}Tc MDP Bone Scan Report (30/4/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post IV Injection of 20mCi of ^{99m}Tc MDP
Revealed:

Unfortunately, Previous bone scan images are not available. Limited comparison done with report dated 24-9-2024 revealed:

- Currently seen low grade non-uniform tracer uptake is seen involving DV12/LV1 ?regressing metastatic deposits, for CT cors correlation.
- The rest of the skeleton is currently scintigraphically free.
- Degenerative changes are noted over the rest of the spine, more marked over the upper cervical vertebrae for CT correlation.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- Follow up bone scan shows ?regressing bone deposits at DV12-LV1 with equivocal changes at upper cervical vertebrae for CT correlation and serial bone scan images.

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology



جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Much Oblived

Mahmoud Akram

Dr. Ahmed Zaher, MD

Patient Name: No'ma Sobhy Shalabi Mohamed	Date: 27/02/2025
Exam: ultrasound ABDOMEN & PELVIS	Age: 10 Year(s)
Medical ID: 3492/2023	Pat. ID: 42376

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: :

- Average in with homogenous echopattern
- No intra hepatic biliary radicle dilatation or focal lesions.
- Portal vein is patent

Gall Bladder: -

- Average distention with normal wall thickness with no stones.
- CBD is not dilated.

Spleen: -

- Average in size displaying homogenous echopattern, no focal lesions.

Kidneys: -

- Are of normal size, well differentiated & no gross stones or back pressure changes or space occupying lesion could be seen.

Para-aortic and pancreatic area: are concealed by gases.

No ascites or free fluid or collection seen.

Urinary Bladder: -

- Minimally distended no stones or focal lesion seen
- **Uterus :** Average size with homogenous echopattern, with no focal lesions.
- No ovarian cysts or adnexal lesions could be detected.
- No abnormal bowel dilatation

Opinion:

- Apart from colonic distension, Unremarkable study

Signature Dr. Sherien Ali El Sharkawy

27/02/2025 2:01 PM

Patient name: Noama Sobhy

Date of examination: 15 May 2025

COLOR DUPLEX EXAMINATION OF THE VENOUS SYSTEM OF LEFT
UPPER LIMB REVEALED :-

- Patent deep venous system along the axillary and brachial veins limb, with no evidence of venous obstruction.
- Patent superficial venous system (Cephalic and basilic veins).
- Extensive upper limb edema ?? cellulitis for clinical correlation.

Conclusion :

- Patent deep and superficial venous system.

Much obliged,
Mona Abo Elenin M.Sc.